

APPENDIX – II

FIRST AID

The immediate treatment given to the injured in an accident or sudden illness before the arrival of the doctor is called first aid.

1 Aim / Objectives:

- a. Save life.
- b. Promote recovery.
- c. Prevent worsening of the condition.
- d. Arrange transport to shift the injured person to hospital

2 Contents of the First Aid box:

1.	Set of splints	:	1	
2.	Roller Bandage	:	10	
3.	Triangular bandages	:	4	
4.	Tourniquet/Rubber bandages	:	2	
5.	Cotton Wool	:	4	
6.	Safety Pins	:	10	
7.	Adhesive Dressing	:	20	
8.	Paracetamol Tab	:	20	
9.	Diazepam Tab	:	10	
10.	Antiseptic Cream	:	1	
11.	Injury Card	:	1	

3 Types of Accident Relief Medical Equipment:

With a view to provide prompt medical aid, the following types of accident relief medical equipments are provided on the railways:

1. Scale- I medical equipments (MRV) are available at nominated stations mentioned in working time table.
2. POMKA (Portable Medical Kit for Accidents) available in all health units, poly clinics, sub-divisional, divisional and zonal hospitals.
3. Scale-II medical equipments (ARME) are available at specified stations in boxes mentioned in working time table.
4. First Aid boxes are provided with Station Masters, Guards of passenger carrying train Guards, workshops, marshalling yards, loco sheds and C&W depots.
5. Special First Aid boxes are provided in all long distance Super fast, Shatabdi, Rajdhani expresses, Dy.SS (Commercial), AC coaches of some of nominated trains.

This box contains 49 items and this is to be utilized for giving medical aid to the passengers by a doctor only.

6. First Aid boxes for Gangmen.
7. First aid boxes available with Station Masters and Guards of passenger carrying trains are to be inspected by DMO once in a month.
8. Keys of the first aid boxes for locations such as stations, marshalling yards, workshops, loco sheds, carriage and wagon depots etc, are kept with the supervisors on duty.
9. The first aid boxes with guards of train carrying passengers will have no keys.

3.1 Utilization of contents of First Aid Box:

a. Splints:

These are used to immobilize and support the fractured limbs.

b. Roller Bandages:

These are used to retain dressings and splints in position

c. Triangular Bandage:

- To retain dressings and splints in position and to immobilize the fractures.
- To support an injured part or in the form of slings.
- To control bleeding.

- To reduce or prevent swelling.
 - Mainly used as bandages like head bandage, chest bandage, shoulder bandage, elbow bandage, hand bandage, hip bandage, foot bandage etc. It is also used as slings like arm sling, triangular sling and cuff and collar sling.
 - d. **Tourniquet / Rubber Bandage:**
It is used to stop bleeding and to stop spreading of poison in case of snake bites. It is used only for hands and legs. It is to be released at regular interval.
 - e. **Cotton Wool:**
It is used to clean/pad the wounds. It is also used to absorb discharges when there is a wound.
 - f. **Safety Pins:**
It is used when Triangular bandages are used for the injured.
 - g. **Adhesive Dressing:**
It is used for minor and superficial wounds only.
 - h. **Paracetamol Tab:**
These are used to relieve minor pains.
 - i. **Diazepam Tab:**
It is a anti-anxiety medication which is used to make the injured person calm to prevent panic
 - j. **Antiseptic Cream:**
It is used to minimize or prevent infection to wounds.
 - k. **Injury Card:**
It is for maintaining the account of the first aid box items.
- 4 Steps to be followed while rendering effective help to a person
- D-Danger
 - R-Response
 - A-Airway
 - B-Breathing
 - C-Circulation of Blood.
- 4.1 Remove the person from the danger; shift him / her to a safe place.
- 4.2 Find out whether the person is responding or not. If he / she responds is Conscious, otherwise unconscious.
- 4.3 If unconscious check ABC; if any failure restore artificially immediately.
- 4.4 AIRWAY:**
- Airway may get blocked due to: Tongue falling back or Foreign body in the airway
- To check airway
- Lay the casualty on his back
 - Open the mouth and see inside
 - If the tongue has fallen back, tilt the head slightly back to bring the tongue to its normal position and open the mouth.
 - If any foreign body is visible inside the mouth it should be removed by inserting two fingers into the mouth carefully.
 - After clearing the airway, the casualty should be put in recovery position.
 - Once air way is clear, breathing starts automatically.
- 4.5 **Breathing**
- Look for the person's chest to rise and fall.
 - Listen for the sounds of inhaled or exhaled air.
 - Feel for exhaled air by putting your finger near the casualty's nose.
 - If no breathing give two inflations of artificial respiration.
- 4.6 **Circulation**
- Feel for a pulse, by gently pressing two fingers (do not use the thumb) on the person's neck between the Adam's apple, or voice box, and the muscle on the side of the neck.

- If you are not feeling the pulse it indicates that heart not functioning
- Restore the heart through cardiac massage
- Pulse rate is 72 times per minute on an average. While noticing the pulse, observe its rate, strength and rhythm.

5 ASPHYXIA (SUFFOCATION)

Definition: When lungs are not getting sufficient fresh air, important organs of the body, mainly the brain is deprived of oxygen. This is a dangerous condition called asphyxia.

5.1 Causes:

- Drowning
- Breathing polluted air
- Pressing of wind pipe (Hanging, Throttling and Strangulation)
- Choking
- Pressure/weight on chest.
- Electric shock
- Some poisons.

5.2 Signs and Symptoms of Asphyxia/Suffocation:

- Low / No Breathing
- Blue colour of cheeks and lips with congestion of face.
- Swelling of veins at neck.
- Unconsciousness.

5.3 General Treatment for Asphyxia:

- Remove cause from casualty or casualty from cause.
- Ensure more fresh air; loosen tight clothing at chest and neck regions.
- Start artificial respiration without wasting time.
- Arrange medical aid.

Note:- (1) Normal Breathing (Respiration) Rate 15 – 18 times per minute.
(2) Normal Heart Beat/pulse rate 72 times per minute.

5.4 Methods of Artificial Respiration:

- Mouth to Mouth Method of Artificial Respiration
- Mouth to Nose Method of Artificial Respiration

5.5 Mouth to Mouth Method of Artificial Respiration:

- Place the casualty on his back on a plain and hard surface.
- Sit by the side of the face and place the hand by the side of chin and tilt the head slightly back so that clear ventilation at throat.
- Cover the casualty's mouth with clean cloth and pinch the nostrils.
- Open your mouth, take fresh air, and cover the lips of the casualty with your lips and blow the air into the mouth of casualty @ 10-12 times per minute.
- While blowing ensure that nostrils are pinched and chest is rising. If chest is not raising it indicates some obstruction in air passage clear the air passage and restart mouth to mouth artificial respiration. This process to be continued till the normal breathing resumed or Doctor arrives which ever is earlier.

5.6 Mouth to Nose method of Artificial Respiration:

This method will be adopted only when mouth cannot be opened due to injury inside the mouth or jaw fracture or the person suffering with fits convulsions. In this method blow the air into the nose of the victim by closing the mouth of the victim with fingers.

5.7 WHEN THE HEART STOPS FUNCTIONING

If the Heart is not working the following symptoms are noticed:

- The face is blue or pale.
- Heart beat and pulse at the root of Neck (carotid pulse) are not felt.